

--SUMMARY--

Decision No. 228/14

26-Nov-2014

R.Nairn

- Second Injury and Enhancement Fund {SIEF} (severity of preexisting condition)
- Board Directives and Guidelines (SIEF) (one hundred per cent relief)

The worker suffered a left knee injury as he descended from a ladder when his left leg buckled and he fell to the ground. The Board granted the employer 75% SIEF relief based on an accident of minor severity and a pre-existing condition of moderate medical significance. The employer appealed, claiming that it should be entitled to 100% SIEF relief.

One hundred per cent SIEF relief may be granted when it is established that a pre-existing condition precipitated or triggered the injury. The example in Board policy is of a worker who sustains injuries after falling as a result of an epileptic seizure. In this case, the accident did not involve a traumatic event; the worker did not slip or trip, and there was no other external or intervening factor which led his leg to buckle. The medical evidence established that the worker had a pre-existing congenital condition of patella alta, which can predispose a person to patellar ligament rupture. The worker's leg buckled, causing him to fall, as a result spontaneous rupture of his patellar tendon.

The Vice-Chair concluded that the prior non-work-related condition was the cause of the injury. The employer was entitled to 100% SIEF relief. The appeal was allowed.

12 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w0115s]
- BOARD DIRECTIVES AND GUIDELINES: Operational Policy Manual, Document No. 14-05-03
- CROSS-REFERENCE: Decision No. 228/14I
- TRIBUNAL DECISIONS CONSIDERED: Decision No. 526/08 (2008), 85 W.S.I.A.T.R. (online) apId; Decision No. 211/11 apId

Style of Cause:

Neutral Citation: 2014 ONWSIAT 2565



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 228/14

BEFORE:

R. Nairn: Vice-Chair

HEARING:

August 12, 2014 at Toronto
Written

DATE OF DECISION:

November 26, 2014

NEUTRAL CITATION:

2014 ONWSIAT 2565

DECISION UNDER APPEAL:

WSIB Appeals Resolution Officer (“ARO”) decision dated
July 8, 2013

APPEARANCES:

For the worker:

Did not participate

For the employer:

Mr. R. Buxton, Paralegal

Interpreter:

N/A

REASONS

(i) Introduction

[1] At the time of the accident under consideration here, the worker was employed as a machine operator in the accident employer's manufacturing business. Born in 1980, the worker started with the employer in about September 2011.

[2] On April 20, 2012, the worker injured his left knee at work. In his Report of Injury/Disease (Form 6) the worker described the incident as follows:

Coming down from a ladder, left foot buckled under me and I fell to the ground. It was later discovered by the doctor in the emergency department that I had torn the patella tendon in the knee.

[3] In their Report of Injury/Disease (Form 7) the employer described the incident as follows:

The worker was walking down the stairs at the shredder when he went to step down on the second last step he experienced severe pain to his left knee and fell to the ground.

The worker did not slip or experience any type of impact to his knee or leg. He was taken to the hospital via ambulance.

[4] In Memo #5 of May 11, 2012, a WSIB (the "Board") Eligibility Adjudicator noted the following after a conversation with the worker:

Call from worker:

- States that on 20Apr12 while coming down a ladder (metal steps similar to stairs on wheels) at the second to last step, his L leg gave out and he experienced severe pain in his L knee
 - o Felt a pop in his L knee and he fell to the ground unable to put weight on his L leg.
 - o States did not slip or trip, unsure as to how it happened.
- States he could not move or walk and ambulance was called and he was taken on a stretcher
- Doesn't remember much, other than waking up on a stretcher.
- Doctor diagnosed with ruptured patella tendon in L knee

[5] As indicated above, the worker sought medical attention and the hospital emergency record of April 20, 2012, provided a diagnosis of "L patella tendon rupture".

[6] On April 26, 2012, the worker had surgery performed on his left knee by Dr. D. Turchin. Dr. Turchin performed a "repair of left patellar tendon". The post-operative diagnosis was "tear, left patellar tendon".

[7] In the course of considering the matter of the worker's entitlement to benefits for his left knee injury, the Board Adjudicator requested a medical opinion and in a memo dated June 4, 2012, Dr. J. Kim concluded:

(...) The prior cholecystectomy is immaterial to the current presentation. The diagnosis of a patellar ligament tear can be compatible with the accident history. The giving away of the knee can be a sign of patella alta (congenital/developmental) which can predispose to this condition.

- [8] As noted in Memo #13 of June 5, 2012, the Eligibility Adjudicator granted the worker entitlement to benefits for his left knee condition and concluded:

I am allowing this disablement unexpected result claim for health care benefits for the L knee and for Loss of Earnings (LOE) that occurred on 20Apr12. The worker reported the accident and sought medical attention immediately on 20Apr12. Clinical and operative notes have been sent to a medical consultant and have determined that the accident history of walking down a ladder and the worker's L knee giving out, is medically compatible with the diagnosis of a ruptured L patella tendon. Although the worker may be predisposed to the injury, the worker was in the course of employment at the time of injury.

(...)

- [9] In 2013, the Board considered the issue of the employer's entitlement to Second Injury and Enhancement Fund ("SIEF") relief from the costs of this worker's claim. In Memo #33 of January 8, 2013, a Case Manager indicated in part:

This machine operator was walking down steps to the shredder when he felt a pop in his left knee. Based on the accident mechanics, I am accepting the accident history is of minor severity. As coming down steps should not cause a disabling injury.

MEDICAL INFORMATION:

The m/c in memo #12A comments that the wkr has patella alta and that this could predispose him to the condition of patellar ligament tear.

Given the m/c comments and the extent of this wkr's injury this would represent a moderate pre-existing condition.

RECOMMENDATION: Based on this review, I am satisfied that the claim was prolonged. Noting a minor accident history and a moderate pre-existing condition, I am allowing 75% SIEF on all costs of the claim.

- [10] In a decision dated January 8, 2013, the Case Manager confirmed that the employer was entitled to 75% SIEF relief from the costs of this worker's claim.

- [11] The employer objected to the quantum of the SIEF relief granted, requesting 100%. The Case Manager agreed to reconsider the issue and in Memo #40 of May 27, 2013, noted:

The a/e rep has provided the ARF as they want 100% SIEF.

They state in their ltr of 15Feb2013 that a pre-existing condition caused the accident and therefore 100% SIEF should be allowed.

Upon further review of this file the accepted acc hx is the wkr was coming down some steps when he went to step down on the second to last step he felt pain to his left knee.

There is no indication that any pre-existing condition caused the accident and therefore 100% is not in order.

- [12] The employer continued to object to the quantum of SIEF relief granted and the matter was eventually referred to an Appeals Resolution Officer ("ARO"). In a decision dated July 8, 2013, the ARO denied the employer's appeal and concluded:

(...)

The employer argued that the worker's pre-existing condition was the sole "cause" of the injury. (...)

Having fully considered the matter, I do not concur with the employer's position. The worker's onset of pain occurred while he was in the process of performing a function of

his work. It can reasonably be argued that had it not been for that process, his knee would not have been exposed to the forces associated with walking down stairs or created a situation where his moderately severe pre-existing condition made him more prone to injury. The pre-existing condition did not cause the 'accident' but it did enhance the resulting injury.

The accident as described can be defined as a disablement caused by an unexpected result of working duties. Therefore, I deny the employer's request for 100% SIEF relief in this claim.

(ii) Issue on appeal

[13] The issue to be determined in this case is whether the 75% SIEF relief granted in this claim is correct.

(iii) Submissions of the employer's representative

[14] The employer agreed to have this appeal considered by means of written submissions and was provided with an opportunity to send in further material in support of their appeal. In a letter dated June 5, 2014, the Tribunal's Office of the Vice-Chair Registrar confirmed with the employer's representative that "you had no further submissions with respect to this appeal."

[15] A review of the case materials indicates that on November 14, 2012, Mr. Buxton wrote to the Board and indicated:

It is requested that this claim be reviewed for the application of SIEF.

Our request is based on the Medical Advisor's opinion that the presence of a congenital condition, identified as patella alta, was the cause of the accident. It is our understanding that this condition can lead to a weakened patellar tendon. There cannot be any doubt that [the worker's] patellar tendon was not in a normal or healthy state, given that all he did was to lift his leg sufficiently to take a step.

In view of the fact that going down stairs is not something that is expected to lead to an injury, much less impairment, we believe that the initiating event was "minor", and the underlying condition was "major".

[16] Mr. Buxton also sent a letter to the Board dated November 16, 2012, in which he advised:

This is a follow-up to our letter of November 14, 2012.

We have since done some research into appeal decisions which may have a bearing on our request for SIEF. This led to the discovery of *Decision No. 211/11*.

On that appeal the circumstances of the injury were virtually identical to those on our claim:

"On December 9, 2006, this then 62 year old welder was working on a step ladder repairing/welding a bin. While carrying out these duties, he was going up and down the ladder. At one point while descending the ladder to retrieve a grinder and, as all the pressure and weight went down on his right leg, he felt immediate excruciating pain as if his right knee popped. He heard a snapping sound and his knee gave way causing him to fall to the ground, landing on his left side."

"As the ARO recounted, while the worker was descending a ladder his right leg gave way. The ARO does not record any incident of the worker slipping, tripping, or, because he was carrying heavy weight, having difficulty with his right leg supporting that weight. Rather, it is clear from the decision under appeal that the worker's right leg simply gave way while he was putting weight on it when he was descending a ladder."

The employer requested and was denied any SIEF, a decision they subsequently appealed to WSIAT, where they were successful. It is our opinion that the reasons for the granting of SIEF are again very applicable to our request:

"Orthopaedic surgeon and Professor of surgery Dr. M. Ford, in an August 11, 2000 report to Mr. Fink, confirmed that he had reviewed all the medical reports in this matter, including the various operative and consulting reports from Dr. Nguyen. Dr. Ford offered the following conclusions in his August 11, 2009 report:

This worker had a spontaneous rupture of his patellar tendon. Most patellar tendon ruptures are secondary to significant trauma in younger individuals. In the absence of that history, then this is quite clearly a ruptured patellar tendon, spontaneous in onset, secondary to degenerative changes. ... There is no evidence here that his patellar tendon rupture was secondary to a distinct work injury. This individual just happened to be at the workplace when his patellar tendon ruptured. Obviously, this could have occurred at home while walking down stairs. His subsequent low back pain is secondary to degenerative aetiology changes that predate his patellar tendon rupture and there is no evidence in the literature that it is directly related to any altered gait that he may have had, secondary to his knee problems ... Degenerative changes have played a major and sole role in his patellar tendon rupture. There is no history of any injury at the workplace. There is no relationship between his occupation and patellar tendon ruptures. [addressing the worker's low back difficulties, which were deemed compensable as a secondary condition] ... [pre-existing degenerative changes] ... obviously played a major role. It is not related to any gait abnormality secondary to his knee problems.

Dr. Ford, of the Orthopaedic and Arthritic Institute, is a well known orthopaedic surgeon as well as a Professor of medicine. His unequivocal opinion is based upon a thorough review of the medical records in this matter. While the ARO concluded that Dr. Ford's conclusions were in some unspecified way "not...consistent with those of the worker's treating physicians", I disagree. None of the treating physicians have offered any definitive opinions on the causation or contribution to the worker's problems. Rather, Dr. Nguyen and the other physicians involved in the worker's care were primarily focused upon treating the worker's problems, and not offering an opinion on causation or contribution.

Moreover, having reviewed the initial report of Dr. Nguyen, the initial history of accident indicates that the worker was not involved in any traumatic event; rather, his right leg "buckled" when he was getting down from a ladder. There is no indication that the worker tripped, slipped, or that there was any other external or intervening factor of chance which led to the worker's leg buckling with him then unfortunately falling. There was also no injury on the worker's left side, where he landed. Rather, the damage was to his right leg.

I thus find Dr. Ford's report consistent with the medical evidence in this matter, as well as objective and reliable. I place much weight on the conclusions offered by this orthopaedic surgeon.

Addressing OPM Document No. 14-05-03, the policy provides as follows:

100% Relief

Full relief of a claim's cost (compensation, health care, permanent impairment) is charged to the SIEF when

- A prior non-work related condition is the cause of the accident, e.g. epilepsy*

Pursuant to the Dr. Ford's unequivocal medical report, I find that the worker, an older gentleman, had a pre-existing condition which was the sole cause of the accident. Dr.

Ford indeed makes clear in his conclusions that the worker's patellar tendon suffered a "spontaneous rupture" which occurred when the worker "just happened to be at the workplace". Dr. Ford concludes that the degenerative changes "played a major and sole role in his patellar tendon rupture", as well as degenerative changes were what caused the worker's subsequent back difficulties.

Accordingly, pursuant to the applicable policy, I thus conclude that the employer is entitled to 100% cost relief. "

As previously mentioned, the circumstances of both claims are virtually identical, other than in the age of the claimants. However, on the other claim it was assumed that due to the worker's age he would have had age-related degeneration in his knee. On our claim a Medical Consultant has suggested that a pre-existing developmental or congenital condition lead to a weakening of the patellar tendon. Given that this is the only possible cause of the tendon rupture, then the provisions of Policy 14-05-03 apply to this claim, as well, and we request 100% cost relief.

(iv) Analysis

[17] Since this worker was injured in 2012, the applicable legislation is the *Workplace Safety and Insurance Act, 1997* (the "WSIA").

[18] Pursuant to section 126 of the WSIA, the Tribunal is required to apply applicable Board policy. In this case, the Board has notified the Tribunal that one of the policies that applies in this case is *Operational Policy Manual* ("OPM") Document No. 14-05-03 entitled "Second Injury and Enhancement Fund". This policy provides in part:

Policy

If a prior disability caused or contributed to the compensable accident, or if the period resulting from an accident becomes prolonged or enhanced due to a pre-existing condition, all or part of the compensation and health care costs may be transferred from the accident employer in Schedule 1 to the SIEF.

Both physical and psychological disabilities are included.

Guidelines

There is no provision in the Act for the Fund to apply to Schedule II employers.

In situations where alcoholism plays a role in the causation of an accident, it is not considered to be a pre-existing condition with regard to the application of SIEF relief.

The objectives of this policy are to provide employers with financial relief when a pre-existing condition enhances or prolongs a work-related disability. It thereby encourages employers to hire workers with disabilities.

Definitions

Pre-accident disability is defined as a condition which has produced periods of disability in the past requiring treatment and disrupting employment.

Pre-existing condition is defined as an underlying or asymptomatic condition which only becomes manifest post-accident.

(...)

100% Relief

Full relief of a claim's cost (compensation, health care, permanent impairment) is charged to the SIEF when

- a prior non-work-related condition is the cause of the accident e.g., epilepsy

- the wearing of an artificial appliance, either work or non-work-related, is the cause of the accident
- a worker has been placed in industry, or is participating in a WCB sponsored Vocational Rehabilitation (VR) program, and suffers
 - an extension of the original disability through minor accident
 - an accident during an on-the-job training program
 - an accident in a trade school or other type of formal training facility

NOTE

“Other type of formal training facility” may also include a Schedule II training facility.

- a worker is participating in a training and/or assessment program approved by the WCB and the work-related disability is the sole cause of an accident to another employee of the training and/or assessing employer
- a worker sustains injuries while on a work station training program.

(...)

SIEF-application to employer costs

Medical significance of pre-existing condition*	Severity of accident**	Percentage of cost transfer***
Minor	Minor Moderate Major	50% 25% 0%
Moderate	Minor Moderate Major	75% 50% 25%
Major	Minor Moderate Major	90%-100% 75% 50%

NOTES

* The medical significance of a condition is assessed in terms of the extent that it makes the worker liable to develop a disability of greater severity than a normal person. An associated pre-accident disability may not exist.

With psychological conditions, the possibility of prior psychic trauma resulting from life experience could be considered as evidence of vulnerability, and justify recommending relief to the employer, even in the absence of pre-existing psychological impairment.

** The severity of the accident is evaluated in terms of the accident history and approved definitions.

Accident History Components

- mechanics (lift, push, pull, fall, blow, etc.)
- position (kneeling, standing, sitting, squatting, bending, etc.)
- environment (lighting, temperature, weather conditions, terrain, etc.)

Definition – “Severity of Accident”

Minor: expected to cause non-disabling or minor disabling injury

Moderate: expected to cause disabling injury

Major: expected to cause serious disability probable permanent disability

*** The percentage of the total cost of the claim transferred to the SIEF.

- [19] As the above-mentioned policy provides, determining the appropriate quantum of SIEF relief to be granted in a particular case involves determining both the severity of the compensable accident and the medical significance of any pre-existing condition.
- [20] With respect to the issue of the pre-existing condition, Board policy provides that where a pre-existing condition enhances or prolongs a worker's recovery, all or part of the compensation and health care costs may be transferred from the employer to the SIEF. The policy also provides that "if a prior non-work-related condition is the cause of the accident, e.g. epilepsy" an employer is entitled to 100% SIEF relief.
- [21] In this case, the employer claims that it ought to be granted 100% SIEF relief because the "only possible cause of the tendon rupture" was the worker's pre-existing developmental/congenital condition.
- [22] In *Decision No. 526/08*, a Tribunal Vice-Chair dealt with a request by an employer for 100% SIEF relief. In that case, the worker had stepped on an airhose at work and then his left knee gave out or buckled and he felt pain. The employer's representative submitted that the simple act of turning or even stepping on a one-half inch airhose could not be considered causative of the worker's left knee problems. The representative submitted that the evidence supported a conclusion that the pre-existing instability in the worker's left knee was the cause of the accident and therefore 100% SIEF relief ought to be granted.
- [23] In *Decision No. 526/08*, the Vice-Chair discussed the provision in the Board policy which suggested that 100% SIEF relief was applicable when a prior non-work-related condition is the cause of the accident. The Vice-Chair noted:

(iv) Conclusions; discussion of the evidence

(a) The request for 100% cost relief: What does the policy mean?

[16] The representative submits that the pre-existing condition "caused" the injury within the terms of the policy. I must first address what the word "cause" means in Board policy when it is used to refer to the first of the three circumstances in which 100% SIEF is granted. For ease of reference, the Policy states:

100% Relief:

Full relief of a claim's cost (compensation, health care, permanent impairment) is charged to the SIEF when

- A prior non-work-related condition is the cause of the accident, e.g. epilepsy [emphasis added]

[17] The word "cause" as used in this provision cannot mean that the pre-existing condition is the "sole" cause of the injury. If a pre-existing condition is the sole cause of an injury, then there is no compensable accident. The worker would not be entitled to benefits. For an accident to be compensable, a work-related injuring process must be a significant contributing factor in the causation of the injury.

[18] Entitlement has been allowed in this case, and that issue is not before me. SIEF is only in issue if entitlement has been allowed. If a worker is not entitled to benefits, an employer does not require cost relief.

[19] Because entitlement has been allowed, this means that the Board has determined that the work-related factors were a significant contributing factor (i.e., a causative

factor) in the injury. I do not have jurisdiction to make a different finding. Initial entitlement has not been appealed.

[20] The reference to the “cause” in this provision of the policy also cannot mean only that the pre-existing condition is one of several causes in the chain of causation. The question of SIEF arises only when there is more than one cause of an injury. It arises when there is a work-related cause, with the result that the injury is compensable, but there also is a non-work related cause. In SIEF cases, a pre-existing condition is also a causative factor because it has enhanced or prolonged the extent of the injury. Therefore if the word “cause” were interpreted to mean that the pre-existing condition need only be one of several causative factors, the provision would apply in every case.

[21] Because the word “cause” cannot be interpreted to mean the sole cause, or “a” cause of the injury, I have considered what the word does mean. I understand it to be a reference to the precipitating or triggering cause of the injury. I consider it intended to distinguish between two types of cases. In the first type of case, the workplace incident is the precipitating cause of the accident. For instance, a worker may slip and fall on a wet floor. However, underlying degenerative disc disease may affect the severity of the injury. In that case, the employer is not entitled to 100% cost relief. It is entitled to SIEF relief based on the Board’s rating schedule for the importance of each of these factors. The underlying condition was not the causal factor that triggered the injuring process.

[22] In the second type of case, it is the pre-existing condition that precipitates or triggers the injury. The Board gives the example of epilepsy. A worker may suffer an injury at work because he has an epileptic seizure that causes him to fall. The injury from the fall may be compensable because workplace factors also contributed. For instance, it is possible that the worker fell off a ladder on which he was working, and suffered more severe injuries than might otherwise have been the case. This is the type of case in which I understand that 100% SIEF applies.

[23] As I read this provision of the policy, the allowance of 100% cost relief is not based on a weighing of the contribution of each of the various factors, as a matter of medical/legal causation, which is the basis for the other rules for apportionment of responsibility set out in the SIEF policy. I read the Board’s stipulation that 100% SIEF applies in the three situations described to be based on policy determinations that are not necessarily directly related to issues of causation.

[24] In this case, I agree with the Board that this employer does not meet the criteria for 100% relief. On any view of these facts, the worker’s stepping on the hose was the triggering event in the sequence of injury. His knee did not give out or buckle, and he did not suffer pain, until he stepped on the uneven surface caused by the air hose. There is no doubt that this was a very minor accident. However, it has been found to be a compensable accident. Based on that finding, it was the stepping on the air hose that initiated the injuring process.

[25] It is not relevant that a worker without a pre-existing condition would not have been harmed by stepping on an air hose. The thin skull rule applies.

[26] Therefore the employer is not entitled to 100% cost relief.

[24] I agree with the reasoning of the Vice-Chair in *Decision No. 526/08* and accept that 100% SIEF relief may be granted when it is established that the pre-existing condition “precipitates or triggers the injury”. The example given in Board policy is that of a worker who sustains injuries after falling as a result of an epileptic seizure.

[25] In support of their position in this appeal, the employer has also referred to Tribunal *Decision No. 211/11*, the facts of which are very similar to the case on appeal and which the Tribunal Vice-Chair described as follows:

[12] As the ARO recounted, while the worker was descending a ladder his right leg gave way. The ARO does not record any incident of the worker slipping, tripping, or, because he was carrying heavy weight, having difficulty with his right leg supporting that weight. Rather, it is clear from the decision under appeal that the worker's right leg simply gave way while he was putting weight on it when he was descending a ladder.

[13] The Form 7 report noted that the worker's leg "gave way and he fell". The worker's Form 6 report, other than listing the date and hour of the accident, does not provide any details of how it occurred. The first report from surgeon Dr. Nguyen dated December 9, 2006 describes the history in this fashion:

On December 9, 2006, the patient was getting down from a stepladder when his right knee buckled and he fell.

[26] In *Decision No. 211/11*, the Vice-Chair granted the employer's request for 100% SIEF relief and indicated in part:

(...)

[16] Orthopaedic surgeon and Professor of surgery Dr. M. Ford, in an August 11, 2000 report to Mr. Fink, confirmed that he had reviewed all the medical reports in this matter, including the various operative and consulting reports from Dr. Nguyen. Dr. Ford offered the following conclusions in his August 11, 2009 report:

This worker had a spontaneous rupture of his patellar tendon. Most patellar tendon ruptures are secondary to significant trauma in younger individuals. In the absence of that history, then this is quite clearly a ruptured patellar tendon, spontaneous in onset, secondary to degenerative changes.... There is no evidence here that his patellar tendon rupture was secondary to a distinct work injury. This individual just happened to be at the workplace when his patellar tendon ruptured. Obviously, this could have occurred at home while walking down stairs. His subsequent low back pain is secondary to degenerative aetiology changes that predate his patellar tendon rupture and there is no evidence in the literature that it is directly related to any altered gait that he may have had, secondary to his knee problems. ... Degenerative changes have played a major and sole role in his patellar tendon rupture. There is no history of any injury at the workplace. There is no relationship between his occupation and patellar tendon ruptures. [addressing the worker's low back difficulties, which were deemed compensable as a secondary condition] ... [pre-existing degenerative changes] ... obviously played a major role. It is not related to any gait abnormality secondary to his knee problems.

(...)

[18] Moreover, having reviewed the initial report of Dr. Nguyen, the initial history of accident indicates that the worker was not involved in any traumatic event; rather, his right leg "buckled" when he was getting down from a ladder. There is no indication that the worker tripped, slipped, or that there was any other external or intervening factor of chance which led to the worker's leg buckling with him then unfortunately falling. There was also no injury on the worker's left side, where he landed. Rather, the damage was to his right leg.

(...)

[21] Pursuant to the Dr. Ford's unequivocal medical report, I find that the worker, an older gentleman, had a pre-existing condition which was the sole cause of the accident. Dr. Ford indeed makes clear in his conclusions that the worker's patellar tendon suffered a "spontaneous rupture" which occurred when the worker "just happened to be at the workplace". Dr. Ford concludes that the degenerative changes "played a major and sole

role in his patellar tendon rupture”, as well as degenerative changes were what caused the worker's subsequent back difficulties.

[22] Accordingly, pursuant to the applicable policy, I thus conclude that the employer is entitled to 100% cost relief.

[27] As indicated above, the facts in the current appeal are very similar to those in *Decision No. 211/11*. Reviewing the mechanics of the incident, there is no suggestion that the worker was involved in any traumatic event; rather, his left leg “buckled” as he was descending a ladder. As the worker acknowledged in Memo #5, he did not trip, slip nor was there any other external or intervening factor which led to his leg buckling and the subsequent fall.

[28] The Board’s Dr. Kim, in his memo of June 4, 2012, confirmed, after reviewing the medical information on file, that the worker's pre-existing/congenital/developmental condition of patella alta can predispose one to a patellar ligament rupture.

[29] In my view, the facts of this case justify the granting of 100% SIEF relief. I find, on a balance of probabilities, that this prior non-work-related condition was the cause of the injury and therefore the employer is entitled to 100% SIEF relief.

DISPOSITION

[30] The employer's appeal is allowed.

[31] The employer is entitled to 100% SIEF relief from the costs of this worker's claim.

DATED: November 26, 2014

SIGNED: R. Nairn